EUNOCURA GENERAL HOSPITAL

Department of Internal Medicine

DISCHARGE SUMMARY

Patient: Asha Rao
MRN: SYN-2026-0147
Age / Sex: 54 years / Female
Admitted: 09 Aug 2026 Discharged: 12 Aug 2026
Ward: Medical ICU, Bed 6

REASON FOR ADMISSION

Fever with cough and breathlessness for four days.

VITALS ON ADMISSION

BP 148/92 mmHg, HR 104 bpm, SpO2 91% on room air, temperature 38.6 C.
After 2 L/min oxygen by nasal prongs: SpO2 96%, HR 98 bpm.

EXAMINATION

Bilateral basal crepitations, right more than left. No pedal oedema.

INVESTIGATIONS

HbA1c 8.4% (raised).
Fasting plasma glucose 168 mg/dL.
Serum creatinine 1.1 mg/dL (within reference range).
Total leucocyte count 14,200 per microlitre (raised).
Chest radiograph: right lower zone consolidation consistent with pneumonia.

COURSE IN HOSPITAL

Started on intravenous ceftriaxone 1 g twice daily on 09 Aug 2026.
Afebrile from 10 Aug 2026. Oxygen discontinued on 11 Aug 2026.

MEDICATIONS ON DISCHARGE

Inj ceftriaxone 1 g intravenous twice daily for 5 days.
Tab paracetamol 650 mg oral every 8 hours as needed for fever.
Tab metformin 500 mg oral twice daily.

DIAGNOSIS

Right lower zone consolidation consistent with pneumonia.
Type 2 diabetes mellitus, suboptimally controlled.

FOLLOW-UP PLAN

Review in medicine OPD after 1 week with repeat CBC and creatinine.
Repeat chest radiograph in 6 weeks.
Return earlier if breathlessness, fever above 38 C, or SpO2 below 92%.

DISCHARGE CONDITION

Afebrile for 48 hours, SpO2 96% on room air, ambulating independently.

Prepared by: Dr. N. Menon, Internal Medicine
Synthetic document for testing. No real patient data.